

A PATIENT'S STORY · BUILT BY DSB

A year ago, I was in the *ICU.*

Five hospitals held pieces of my health record. None of them could see the whole picture.

Not even me. So I built the platform that could.

It began with a *hospital bed.*

About a year ago I was critically ill and admitted to intensive care.

The kind of sick where every lab result matters, and where they arrive faster than anyone can make sense of them.

I left with my life. And a *paper folder*.

That hospital had no single source of truth for health data.

When I was discharged, my lab results were simply handed to me. **On paper.**

Five hospitals. Five *paper trails*.

It was the same everywhere I went for tests or admission.

- **5 hospitals** over one health journey
- **0 shared records** between any of them
- **1 patient** left carrying all the paper

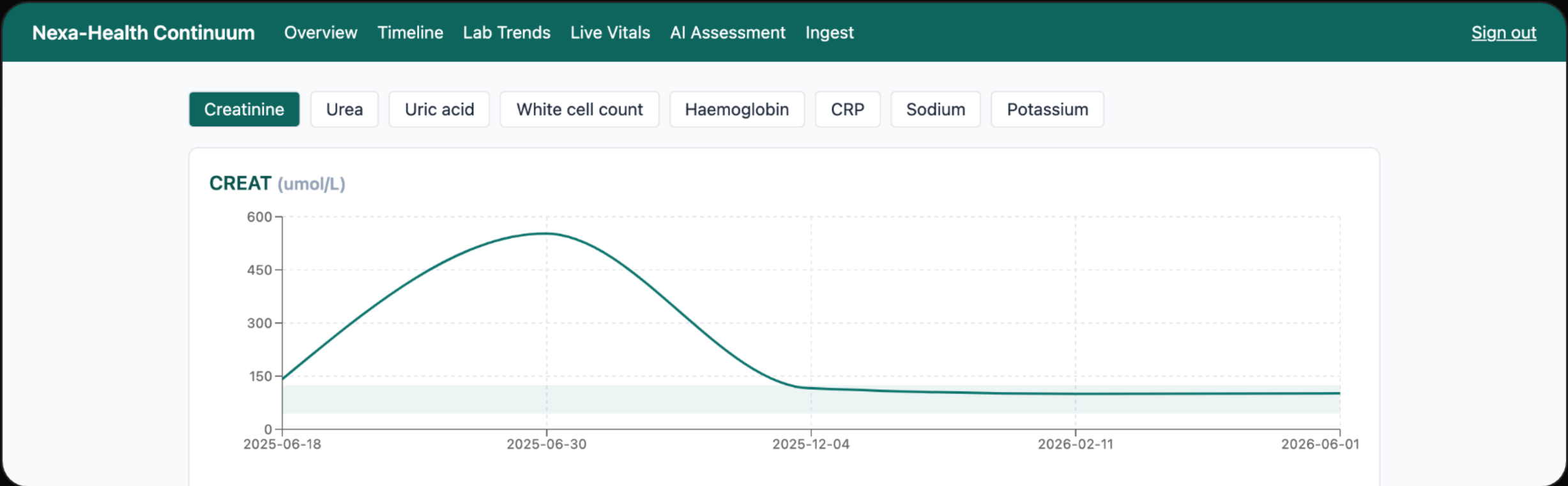
No one could see the *whole picture*.

When your history lives in five places, every clinician sees a slice.
Decisions get made on fragments.

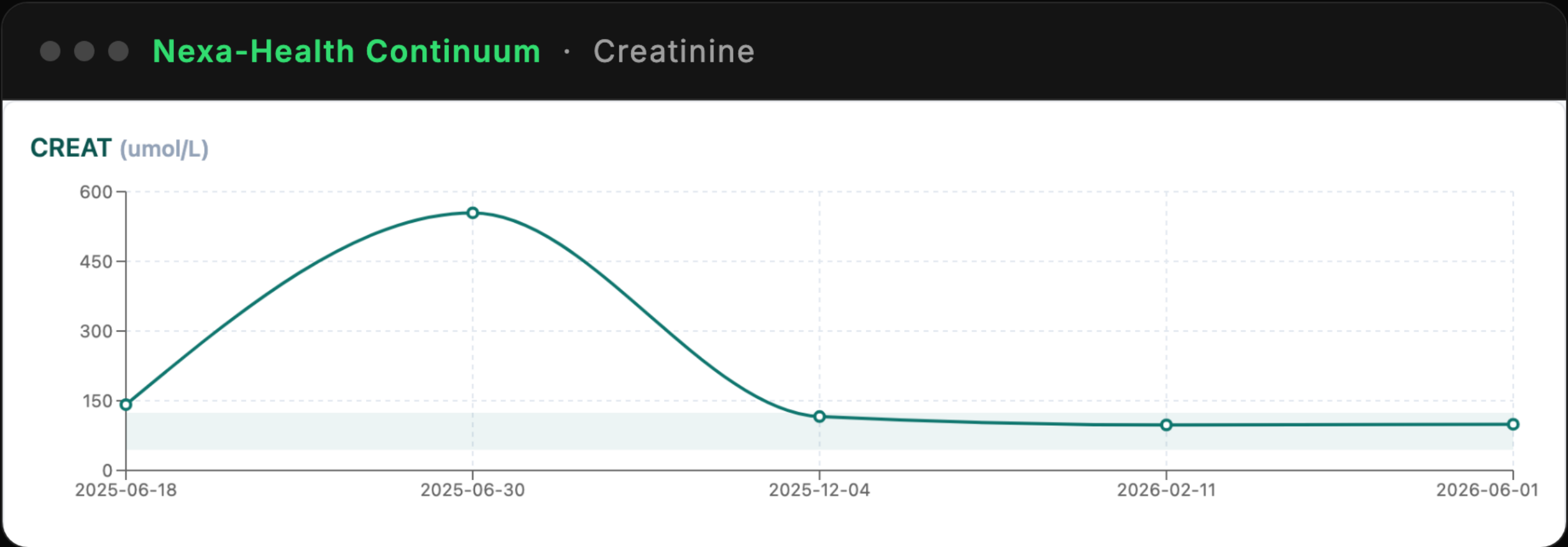
The most dangerous gap in my care was not a missing drug. **It was a missing view.**

So I built *Nexa-Health Continuum.*

One timeline. Every provider. Every result, from every hospital, on a single screen.

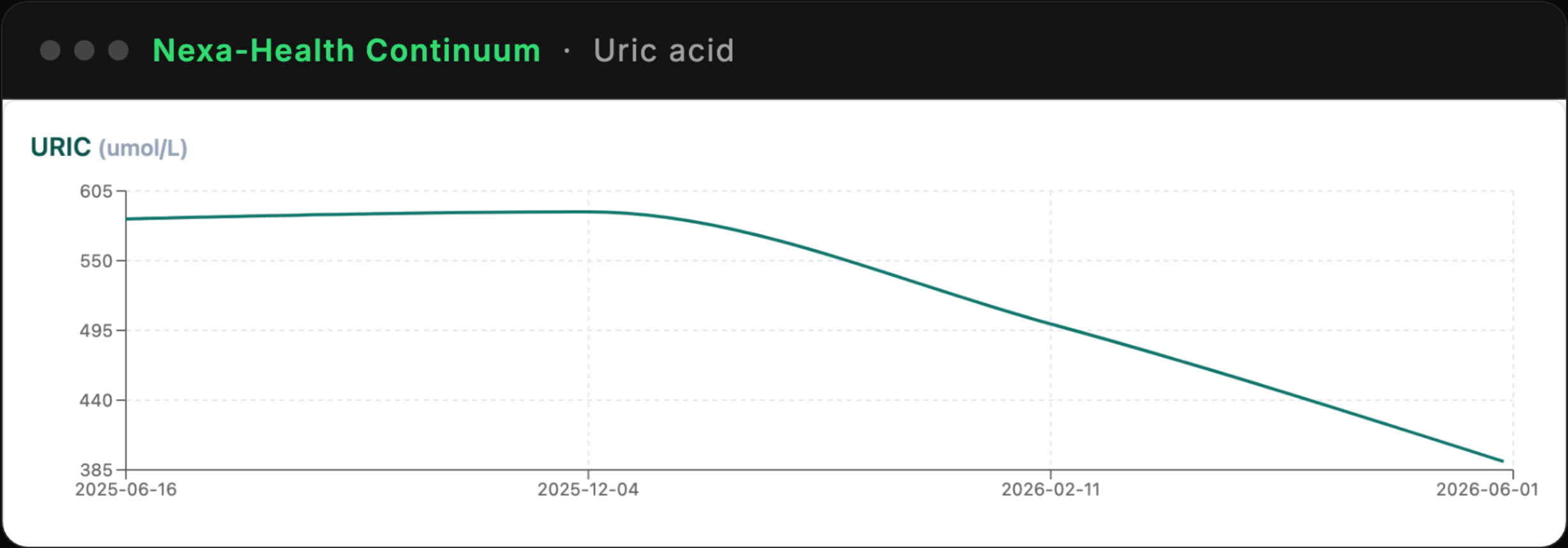


My kidneys, *over a year.*



Creatinine peaked at **554 $\mu\text{mol/L}$** in the ICU. Critical. A year later it sits at **99**. Normal. For the first time, I can see the whole arc.

Recovery you can *actually* see.



Uric acid sliding from **583** back into range. CRP that screamed at **96 mg/L** in the crisis, sixteen times the ceiling. Five hospitals of labs, finally on **one axis**.

AI on the keys. Domain in my *head*.

Built with Claude and a crew of specialist AI agents, on five years of public-health work.

- Vision AI reads scanned reports a clinician verifies every one
- Any provider's results unified to one unit, one scale
- Speaks FHIR and DHIS2 it plugs into real health systems
- Every change passed code review, QA and a governance gate

Your records. *Protected by design.*

Privacy and security were built in from the first line, aligned with modern data-protection principles.

- **De-identified by default** real records stay private
- **Role-based access** + strict tenant isolation
- **Encrypted in transit** + an append-only audit trail
- **AI drafts, a clinician signs off** nothing auto-finalizes
- **8-layer AI-governance standard** with a DPIA before any live use

Every patient deserves *one* source of truth.

I had to nearly lose my health to feel the cost of scattered records. No one should.

A continuous, portable, **patient-owned** health story. That is what I am building toward.

YOUR MOVE

Building in health, or building *from scratch*?

I share this build-in-public journey, one shipped product at a time.
Follow along.

Founders and health teams building something that has to be trusted?
I'd be glad to talk.